

Abstract

Background: Mayer-Rokitansky-Küster-Hauser (MRKH) syndrome is a rare congenital condition (1 in 4,500–5,000 female births) characterized by uterovaginal agenesis in individuals with a 46,XX karyotype. Diagnosed in adolescence, it carries substantial psychosocial burden that remains insufficiently synthesized.

Methods: Scoping review (Arksey & O'Malley; PRISMA-ScR) of PubMed, Scopus, and EBSCOhost databases, January 2019–March 2026.

Results: 39 studies were provisionally included. Psychosexual and relational challenges were most frequent (n = 25), followed by psychological distress (n = 22), reduced QoL, body image and self-esteem (n = 20), and depression and anxiety (n = 12). Coping was documented in 14 studies; healthcare gaps in 10.

Conclusion: MRKH-related psychosocial burden is substantial yet under-integrated into care. *Results updated from the submitted abstract after final screening.*

Introduction, Objective, and Research Question

Introduction

MRKH syndrome is congenital absence of the uterus and upper vagina in individuals with a 46,XX karyotype and functional ovaries, affecting about 1 in 4,500–5,000 female births.^{1,2} It is usually diagnosed in adolescence during evaluation for primary amenorrhea,¹ a critical period for identity and social development. The diagnosis affects fertility, sexuality, and self-image, and psychosocial counseling is recommended as part of care,¹ yet mental health outcomes remain insufficiently integrated into practice.

Objective

To map mental health outcomes, coping mechanisms, and healthcare system gaps reported in studies of individuals with MRKH published 2019–2026.

Research Question

What mental health outcomes and coping mechanisms are reported among individuals with MRKH syndrome, and what healthcare system gaps exist?

Methods

METHOD	DESCRIPTION
Design	Scoping review following Arksey & O'Malley's framework; ³ reported per PRISMA-ScR ⁴
Search	PubMed; Scopus; EBSCOhost (MEDLINE, CINAHL, APA PsycInfo, Women's Studies International). Jan 2019–Mar 2026; run Mar 29, 2026
Eligibility	Primary quantitative, qualitative, or mixed-methods studies reporting psychological outcomes or coping in MRKH; surgical/fertility-only studies excluded
Screening & Analysis	516 records; 324 screened; 46 assessed; 39 included. AI-assisted screening and abstract-level charting (verification in progress); frequency counts and thematic grouping

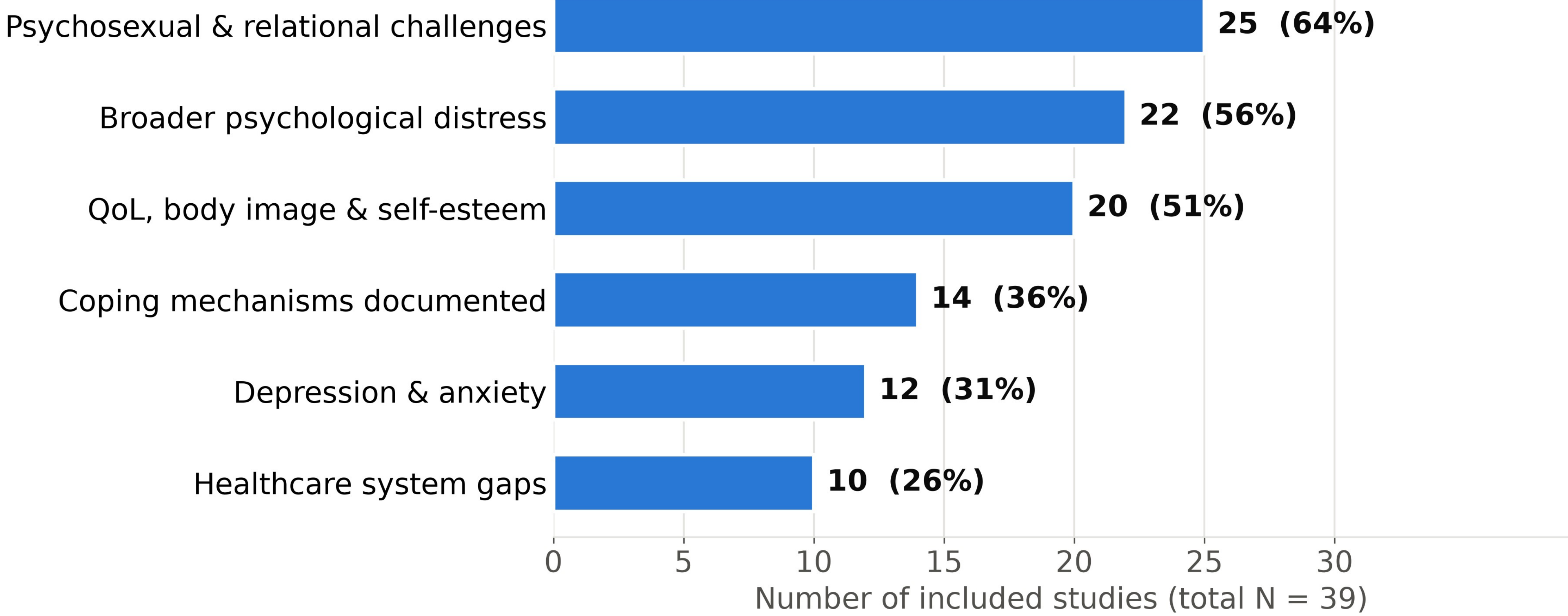


Figure 1. Frequency of Outcome Domains Across Included Studies (N = 39); studies could report more than one domain

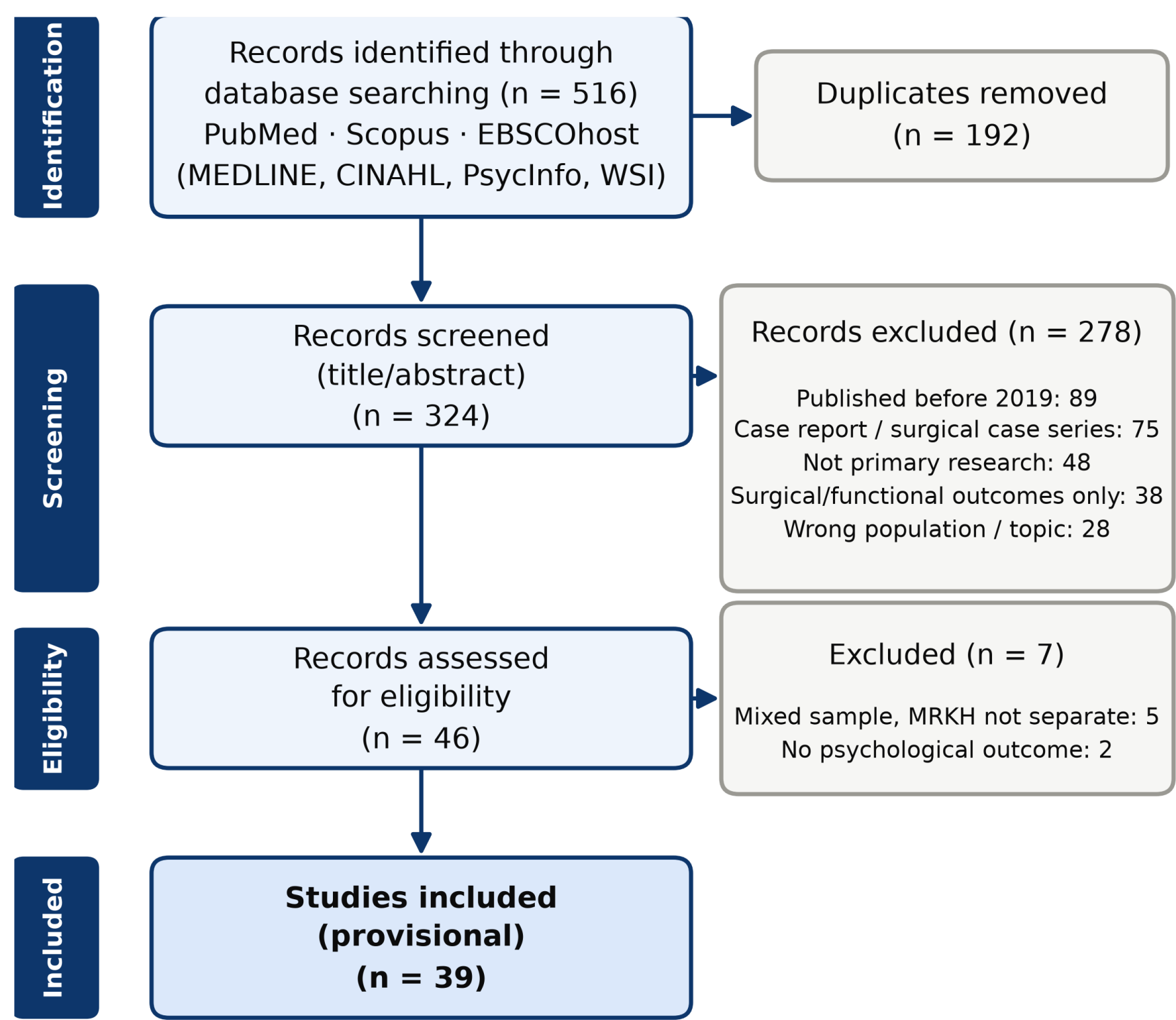


Figure 2. PRISMA-ScR Flow Diagram of Study Selection (provisional)

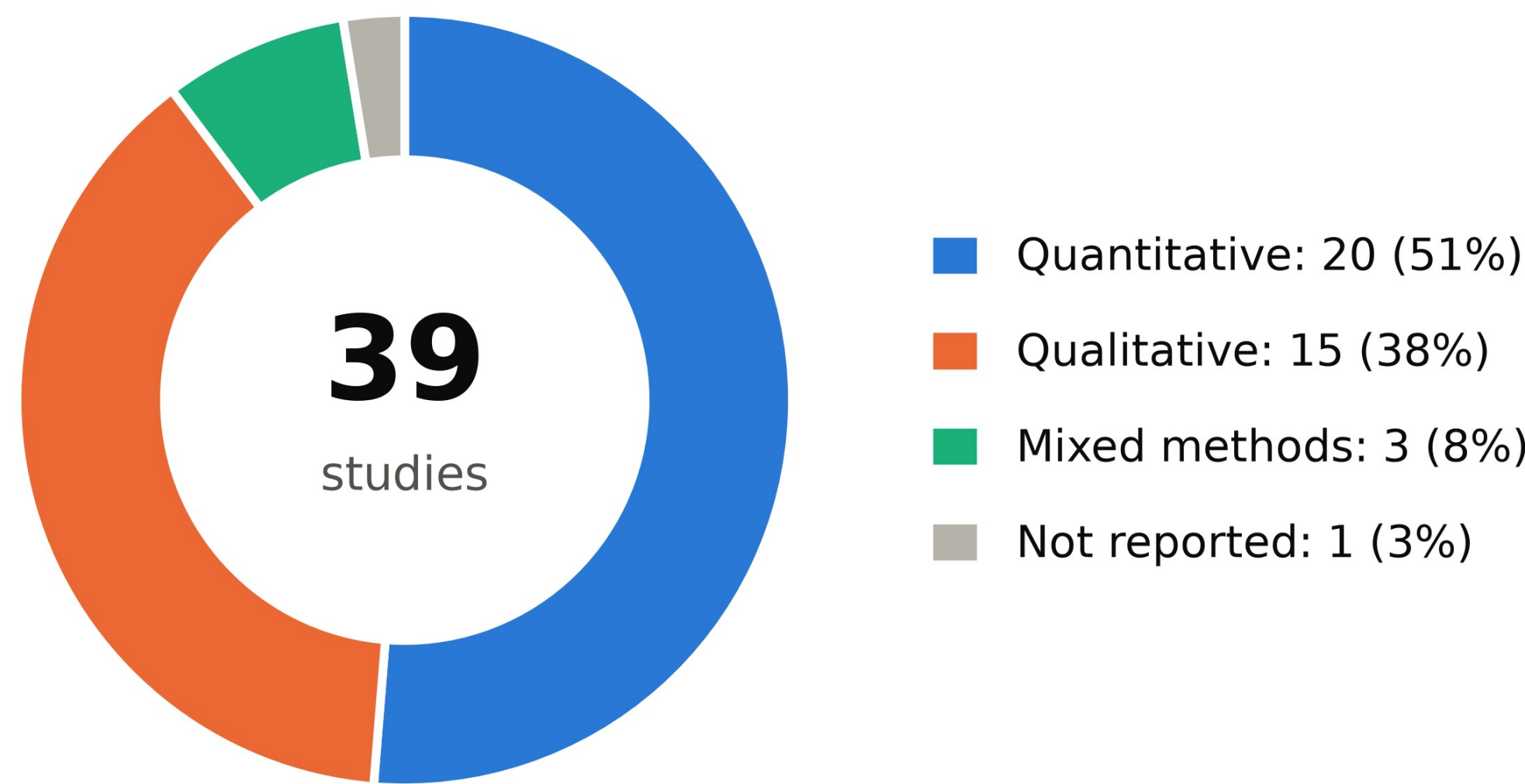


Figure 4. Study Design Distribution (N = 39)

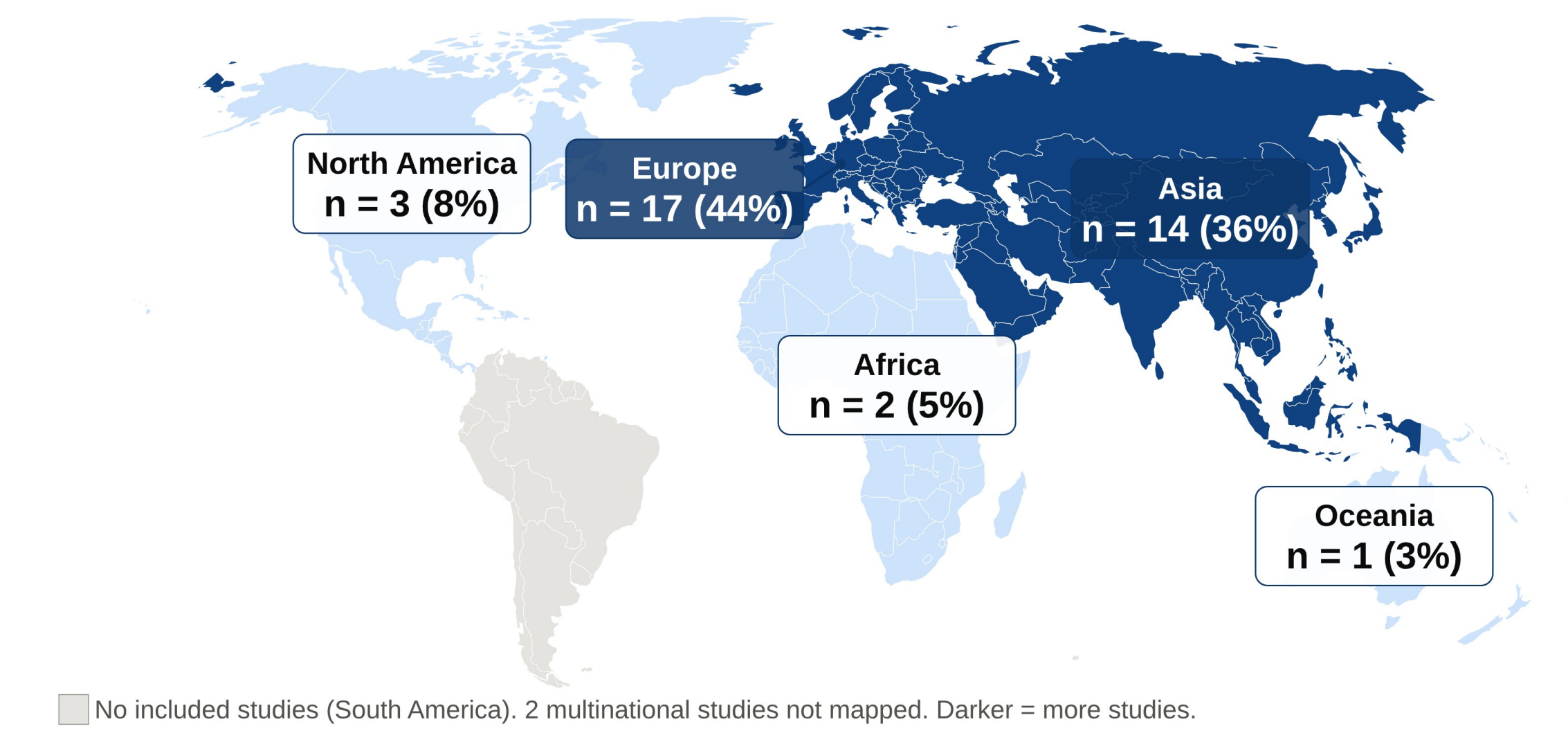


Figure 3. Geographic Distribution of Included Studies (N = 39)

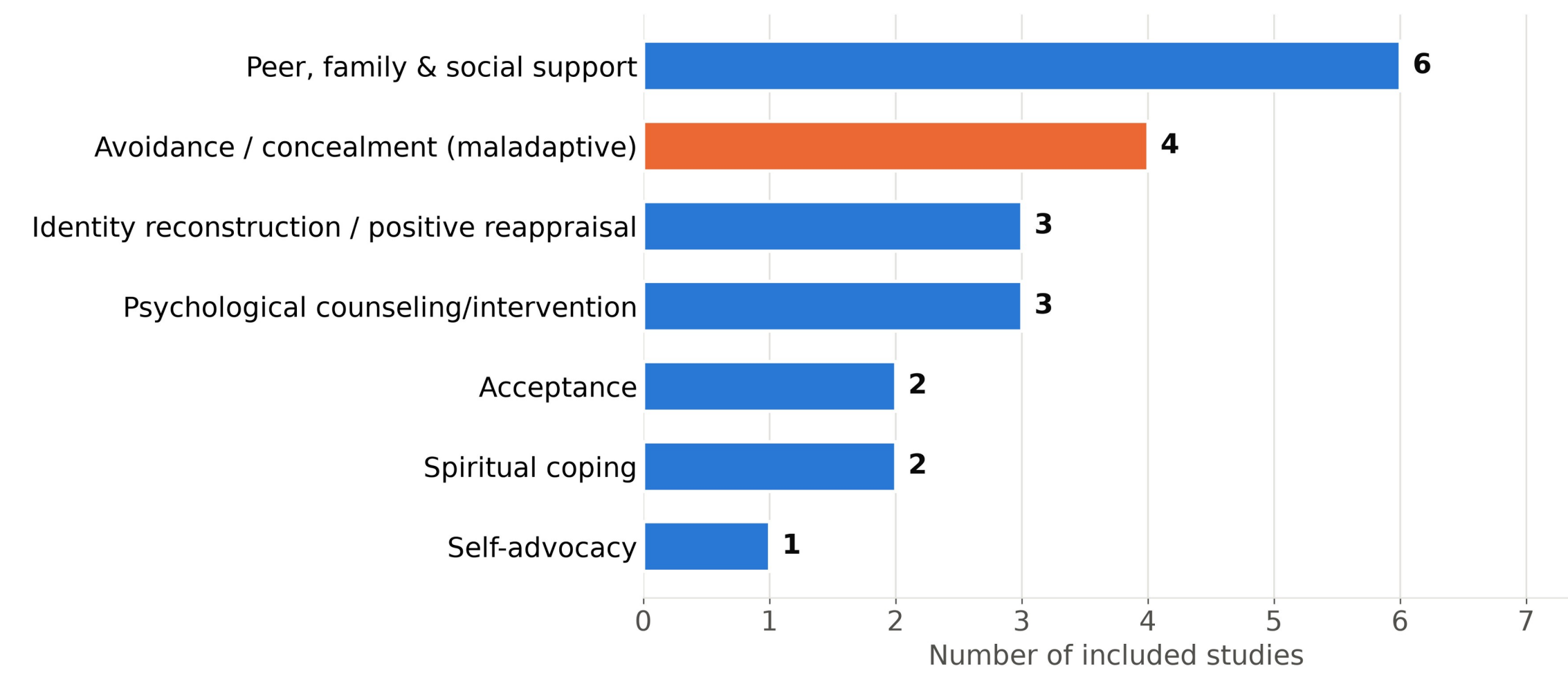


Figure 5. Coping Mechanisms Documented (14 studies)

Results, Discussion, Conclusion, and Limitations

Results

Of 516 records, 324 were screened and 39 studies included: 20 quantitative, 15 qualitative, 3 mixed-methods, 1 not reported. Studies came from Europe (17), Asia (14), North America (3), Africa (2), and Oceania (1); none from South America. Psychosexual and relational challenges were most common (64%); depression and anxiety were measured least (31%), with mixed findings.

Discussion

Findings align with earlier evidence⁵⁻⁷ and a prior review highlighting sexual esteem and genital image.⁸ Coping relied on peer support and self-management; only 3 studies tested interventions. Healthcare gaps included providers' limited knowledge of MRKH and insensitive communication at diagnosis.

Conclusion

Psychosocial burden in MRKH is substantial yet under-integrated into care; multidisciplinary, mental health–inclusive care is needed.

Limitations

- Provisional: AI-assisted screening and abstract-level charting pending full-text verification
- Review limited to 2019–2026; heterogeneous designs and measures
- Few studies from Africa or Oceania; none from South America

Public Health Implications and Future Work

Public Health Implications

- Integrate psychosocial support and mental health screening at diagnosis
- Build multidisciplinary teams with psychology and social work
- Pair vaginal lengthening treatment with psychosexual counseling
- Train providers in sensitive, non-stigmatizing communication

Future Work

- Longitudinal studies with validated measures
- Trials of psychosocial interventions in routine care
- Research in underrepresented and low-resource settings

References

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- Full list of the 39 included studies available on request.
- Acknowledgements:** Mentorship by Dr. Paul Okojie and Dr. Robyn Anderson. No external funding.